NEWS2 Observation and Escalation Protocol

Reference	NUR-OBS-01 v5
Owner	Chief Nursing Officer
Approved by	Quality and Patient Safety Committee
Effective date	04/05/2026
Next review	04/05/2029
Applies to	All clinical areas of the Centre Hospitalier Saint-Aubin

1. Scoring

The National Early Warning Score 2 (NEWS2) is the sum of seven parameters, each scored 0 to 3, plus 2 points if the patient is receiving supplemental oxygen.

Score	3	2	1	0	1	2	3
Respiration rate	8 or less		9-11	12-20		21-24	25 or more
SpO2 (scale 1, %)	91 or less	92-93	94-95	96 or more			
Air or oxygen		oxygen		air			
Systolic BP (mmHg)	90 or less	91-100	101-110	111-219			220 or more
Pulse (per min)	40 or less		41-50	51-90	91-110	111-130	131 or more
Consciousness				Alert			Confusion, V, P or U
Temperature (C)	35.0 or less		35.1-36.0	36.1-38.0	38.1-39.0	39.1 or more	

2. Clinical response

Aggregate NEWS2	Response	Minimum frequency of observations
0	routine monitoring	every 12 hours
1 to 4	ward-based response: registered nurse decides	every 4 to 6 hours
3 in a single parameter	urgent ward-based response	as directed
5 or 6	urgent response: urgent review by a clinician within 30 minutes	at least hourly
7 or more	emergency response: immediate assessment by the critical care outreach team	continuous monitoring

3. Recording

A score of 3 in any single parameter takes precedence over the aggregate band and triggers at least an urgent ward-based response. The aggregate score is calculated at every set of observations from the chart values using the table above. The time of the escalation call and the name of the clinician who answered are recorded in the notes.

4. Governance, audit and training

This document is controlled. Printed copies are uncontrolled: the current version is held on the hospital document management system and a printed copy must be checked against it before use.

The document owner reviews the content at least every three years or earlier when national guidance, a serious incident or an audit finding requires it. Interim changes are approved by the relevant committee chair and recorded in the revision history.

Compliance is audited by the clinical audit department on a rolling sample of records. Results are reported to the quality and patient safety committee, which agrees actions with the ward managers and monitors their completion.

Staff who identify a deviation from this document, or a situation it does not cover, should discuss it with the senior clinician on duty and report it through the incident reporting system. Reporting is a learning tool and not a disciplinary process.

5. Revision history

Version	Date	Author	Summary of change
1.0	04/05/2021	Chief Nursing Officer	First issue
2.0	04/05/2023	Chief Nursing Officer	Triennial review; format harmonised
3.0	04/06/2025	Chief Nursing Officer	Tables updated after clinical audit
current	04/05/2026	Chief Nursing Officer	Review and re-issue

Annex A. Implementation notes

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Data protection: any patient information handled under this document is processed in accordance with the data protection policy of the hospital. Access to records is limited to staff involved in the care of the patient.

Equality and diversity: this document has been assessed for its effect on patients and staff with protected characteristics. Reasonable adjustments, interpreters and accessible formats are available through the patient experience team.

Where this document refers to a role (for example the ward pharmacist or the senior nurse), the post-holder on duty carries out the task or delegates it to a suitably trained colleague who remains accountable to them.

Training on this document is part of the induction programme for clinical staff and is refreshed through the annual mandatory training cycle. Agency and locum staff receive a short briefing at the start of their first shift.

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Annex B. Implementation notes

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